

DFC Emergency Response Initiative - Strategic Proposal & MOU Framework

Draft v0.2 - for core-team review and ratification Doctors Foundation for Care (DFC) | Emergency Response Initiative (ERI) A DFC-owned initiative. Partners addressed: NEMSAS (National Emergency Medical Service & Ambulance System) and the NHSRII / SWAP Office. Prepared following the 22 April 2026 engagement and the subsequent core-team strategy session.

Status of this draft. Discussion document, not a final submission. Costed figures live in the companion [Costing Model](#) and are a transparent planning model with every assumption flagged for the Finance pillar to validate (Nigeria has no reliable published EMS unit-cost dataset, so we build bottom-up). The ambulance-aggregation precedents in Section 4 are drawn from a dedicated, adversarially-verified research pass (sources cited inline). Circulate, mark up, and return comments before this goes to Dr. Muntaqa and Dr. Doubra.

0. The one-sentence position (read this first)

Everything below hangs off a single positioning statement that resolves the tension the group keeps circling. NEMSAS asked DFC to *advocate and aggregate*; the core team concluded DFC's strength is *clinical delivery and standards*. The resolution:

DFC is the private-sector engine for emergency care. It sets the standards AND implements them; it builds the workforce; it pools private-sector resources into a single coordinated block; and it makes NEMSAS's mandate executable on the ground. It advocates only where advocacy unlocks its own delivery: tariff authority, legal backing for the trauma fund, and a standalone EMS Act.

This keeps DFC out of pure politics (the core team's worry), honours Dr. Doubra's three asks (which are mostly about *coordination and delivery capacity*: SOPs, 112, NCC), and gives funders a fundable object: a system that works, not a campaign.

1. What DFC actually does - the mandate in four verbs

The whole proposal is DFC doing four things, in order of distinctiveness:

#	DFC's role	What it means concretely
1. Set standards AND implement	Author the SOPs, triage algorithms, ambulance protocols and clinical stabilisation standards - and then <i>run</i> them in the pilot, not hand them over and walk away. Standards without implementation is consulting; DFC does both.	
2. Develop the workforce	Train and certify EMTs, paramedics, 112 operators, ambulance crews, community responders and trainers. Ambition: DFC becomes an accredited EMS training institution (Section 5).	
3. Pool resources	Aggregate the fragmented private ambulance sector into one credentialed block (Section 4); pool funding, equipment, hospital capacity and clinical expertise into a coordinated whole.	
4. Leverage private-sector contributions	Convene CSR, corporate fleets, private hospitals, HMOs, telcos, banks and oil & gas into the initiative - the concrete meaning of "the private-sector coalescing body for emergency care."	

Advocacy is a fifth, deliberately narrow, role - only where it unlocks the four above.

2. The strategic wedge: start with road-traffic-accident (RTA) trauma

We will **not** propose to cover the whole patient journey for every emergency at once. We pick one vertical, build it end-to-end, prove it, and scale. This is our own conviction, and it is what makes the initiative fundable and measurable.

Reason RTA is the right wedge	Detail
Measurable	FRSC publishes crash data - 5,421 road deaths in 2024 (verified) - giving us a denominator and a baseline.
Politically clean	No one defends road deaths. No constituency to fight.
Sharp metric	The "golden hour" gives a defensible, time-bound outcome target.
Architecturally generic	The chain we build for RTA (call → triage → dispatch → nearest ambulance → stabilisation → right hospital) is the spine for every other emergency. Narrowing the use-case does not narrow the system.

Reason RTA is the right wedge	Detail
Funder-ready	Catalytic funders buy startable, measurable pilots. Trauma is the cleanest.
Real burden	A verified Nigerian study found 86% of road-crash victims faced catastrophic health expenditure . The equity case writes itself.

We are not abandoning the wider vision. We are building its spine first.

3. The delivery stack and the tiered offer

DFC offers NEMSAS a five-layer **delivery stack**, each layer mapping to one of Dr. Doubra's formal asks, re-cut from "advocacy" into "execution":

Layer	What DFC does	Maps to	Detail
Ambulance aggregation	Curate, credential and manage a private + public ambulance pool; present to NEMSAS as a <i>single contracting unit</i>	The "aggregate and present as one unit" ask	Section 4
Workforce / training	Accredited EMT, paramedic, operator, responder and trainer programmes	Capacity & standards gap	Section 5
Standards & SOPs	Triage algorithms, call-centre SOPs, ambulance protocols, QA framework	Ask #2	Section 6
112 / NCC strengthening	Operator training, AI decision-support, supervision & QA of the 112 line	Ask #2 + #3	Section 6
Advocacy (narrow)	Standalone EMS Act, embedded tariff authority, legal basis for the trauma fund	Ask #1	§0

The **tiered offer** lets government and the group choose *depth* without renegotiating *scope*:

- **Tier 0 - Standards & coordination only.** SOPs + 112 operator training + ambulance-credentialing framework. Cheapest, fastest, lowest-risk. **DFC can do this with or without government money.**
- **Tier 1 - Coordinated pilot.** Tier 0 + a live aggregated ambulance network + dispatch tech in the pilot LGAs, measured against golden-hour metrics. ← **Recommended pilot.**
- **Tier 2 - Funded trauma response.** Tier 1 + the "first 48 hours free" trauma fund operating in-pilot. The scale-up the financing architecture switches on.

We recommend the proposal *leads with Tier 1* and presents Tier 2's trauma fund as the statutory-funding switch-on, keeping the core ask grant-fundable and de-risked.

4. Flagship: DFC as the ambulance aggregator

This is the most concrete, most distinctive, and most fundable thing DFC can offer NEMSAS - and it was NEMSAS's own ask, so there is pull, not push.

4.1 The problem DFC solves

NEMSAS needs ambulance capacity but **cannot credential, supervise, dispatch or pay hundreds of fragmented private operators one by one**. The Nigerian private ambulance sector - private hospitals, standalone ambulance companies, logistics/health-tech players (LifeBank, ERA-type operators), faith-based and corporate fleets - is fragmented, under-utilised, and has no single counterparty government can contract.

Fragmentation is not just an inconvenience - it is a hard structural ceiling. A formal model of decentralised ambulance systems shows that when many independent providers operate with no central coordination, the *maximum achievable coverage* (the probability that any given call is served by at least one ambulance) is capped at roughly **71.5%, no matter how profitable the system is** (Springer, 2024). Aggregation is the only way past that ceiling. Continent-wide, the same fragmentation means an estimated **95% of Africa lacks a centralised EMS system** (RSM, 2023).

4.2 The model: asset-light aggregation, not fleet ownership

This is the India "Dial 108" PPP **inverted**. GVK EMRI is one operator a government funds to *buy and run* thousands of ambulances. DFC instead **aggregates capacity that already exists** and presents it as one credentialed, dispatchable, accountable block. The capital saving is the whole point: DFC pools idle private vehicles rather than purchasing a fleet.

How the single block works: - DFC stands up a **managed network** (a framework / panel structure). Operators sign one participation agreement covering vehicle standards, crew certification, **tariff acceptance**, response-time SLAs, data-sharing, reporting, and deactivation-for-non-compliance. - **NEMSAS signs one framework contract with DFC**. DFC sub-contracts and supervises the pool. This is exactly how India's 108 system is governed: **a single MOU per state government** with one operating entity, rather than the government managing many operators - MOU values that have ranged from ₹50 crore/yr (Rajasthan) to ₹252 crore over five years (Gujarat) (NHSRC India). - DFC runs credentialing, dispatch (nearest available unit, public or private), QA / ratings, and - critically - **payment reconciliation**.

4.3 The business model (how it sustains itself)

Four revenue lines make the aggregator self-funding:

1. **Take-rate** on each reimbursed dispatch (a coordination fee on the tariff) funds QA and the platform.

2. **Subscription / membership.** Kenya's Flare charges roughly **KES 4,000 (~€33) per subscriber per year**, with discounts via group membership and insurance risk-pooling (RSM, 2023). Operators also pay to access the platform and its demand.
3. **Payment aggregation / clearinghouse.** This is the line that de-risks operator participation, and **the statutory rail already exists in Nigeria**. Under the BHCPF, **5% of the Fund is allocated through the EMT Gateway to NEMSAS, which pays for emergency ambulance services delivered by accredited private AND public providers**. The law already anticipates private operators as the delivery layer, reimbursed from a central government fund. DFC's clearinghouse routes that reimbursement (plus insurer payments) *through* to operators, solving their single biggest fear: payment delay. *(The exact 5%/EMT-Gateway governance detail should be confirmed at source with NEMSAS; see Costing Model evidence base.)*
4. **B2B cover.** HMOs, insurers and corporates buy guaranteed emergency response routed through the pool. Flare's model shows insurance integration as a core reimbursement channel, not an afterthought.

The flywheel: aggregation raises utilisation of idle private ambulances → operators earn more → more join → denser coverage → faster golden-hour response → more demand and funding.

4.4 Value proposition

To NEMSAS / government	To operators
Onboard private capacity at speed	Demand and higher utilisation of idle vehicles
A single point of supervision and contract	Guaranteed, fast reimbursement (DFC clearinghouse)
Standardised tariff, SLAs and reporting	A credentialing badge and training
Surge capacity and national scalability	Access to BHCPF / insurer payment rails

4.5 Risks the model must address head-on (credibility with government)

- **Payment delay killing participation** → DFC settlement float / escrow and fast reconciliation.
- **Quality control across independent operators** → enforceable SLAs, ratings, and deactivation.
- **Adverse selection / cherry-picking** → minimum-service obligations in the participation agreement.
- **Dispatch disputes** → a transparent nearest-available-unit algorithm.

4.6 Precedents - the model is proven, not theoretical

Every claim below survived adversarial 3-vote verification (sources cited).

Flare / Rescue.co (Kenya) - the closest analogue to what DFC proposes. Flare owns no ambulances; it aggregates existing private providers, assessing each for service, personnel and

equipment quality, onboarding them via **formal service-level agreements**, and providing dispatch technology, GPS devices, triage training and hospital-referral dashboards (RSM) (PMC). By 2025 it had aggregated **800+ public and private ambulance providers** onto one dispatch platform, coordinating **47,000+ emergency responses across all 47 Kenyan counties** and linking **2,000 health facilities** (PMC) (Oxford OODH). Rescue.co reports urban response times falling from a ~160-minute baseline to under 15 minutes (operator-reported; magnitude not independently confirmed).

India 108 / GVK EMRI - the single-operator PPP at national scale. One private entity is contracted by state governments to run the *entire* fleet: **9,655 ambulances, 66,472+ employees, 17 states + 1 Union Territory, ~1.02 billion people covered** (EMRI). Crucially for the financing case, **state governments fund ~95% of operating cost** (100% of capital in Gujarat and Rajasthan), the operator's own contribution falls to ~5% by year two, and the service is **free and toll-free to patients** (NHSRC India). This is the public-funds-the-recurring-cost, private-runs-the-system structure DFC is proposing.

Emergency Response Africa (ERA), Edo State - the Nigerian proof point. A government EMS partnership (Edo State govt, University of Benin Teaching Hospital, Edo State Health Insurance Scheme), piloted from **5 March 2024 for 15 months**. It integrates independent third-party medics, ambulance providers and hospitals into one system, run from a **24/7 Command Center** that dispatches the closest available unit, linked to **112 (federal) and 739 (Edo)** via three apps (Dispatch, Responder, Hospital). The pilot trained **170 Community-Based First Responders across 3 LGAs** and stationed **4 ambulances at PHCs**, targeting a **10-minute** access window (ERA). This is the responder-on-foot + aggregated-ambulance model, already operating in Nigeria.

Credentialing at scale - a ready template. The **CAAS Ground Vehicle Standard** sets minimum vehicle requirements and enforces them through a **certify-then-mark compliance symbol**: operators who do not meet the standard cannot display the mark and are excluded (CAAS). DFC can adapt this exact mechanism to credential a fragmented fleet at speed.

Lever for DFC	What the precedent proves
Asset-light aggregation works at scale	Flare: 800+ providers, 47 counties, no owned fleet
Single-counterparty contracting	India 108: one MOU per state, not many operators
Public funds the recurring cost	India 108: government ~95% of opex; patient pays nothing
The Nigerian model already runs	ERA Edo: aggregated dispatch + responders, live since 2024
Credentialing is a solved problem	CAAS GVS certify-then-mark standard
The statutory payment rail exists	BHCPF EMT Gateway reimburses accredited private + public providers

5. Human-resource development: DFC as an accredited training institution

NEMSAS named **standards-setting, supervision and workforce development as the current gap**. This is where DFC's identity as a doctors' institution is decisive - and where a durable, revenue-generating pillar lives.

5.1 The ambition

DFC pursues accreditation as an EMS training institution, issuing recognised certifications across the full emergency workforce:

Programme	Who it trains
EMT (basic → intermediate)	Ambulance crews, first responders
Paramedic up-skilling	Advanced pre-hospital providers
112 / call-centre operator certification	Dispatchers, triage operators
Responder-on-foot / community first responder	Hard-to-reach community responders (bleeding-control, basic trauma)
Hospital trauma-team training	ED nurses and doctors at pilot hospitals
Train-the-trainer	The multiplier that lets the system scale nationally

5.2 Why this is strategic, not a side activity

- **It closes the exact gap NEMSAS named** - standards and supervision - by making DFC the body that *certifies* the workforce to those standards.
- **It is a sustainability pillar.** Training fees, government and donor-funded workforce contracts, and certification fees are recurring revenue that funds the rest of the initiative.
- **It compounds the aggregation model.** Every credentialed operator in the pool is trained and certified by DFC, so standards are enforced through the workforce, not just on paper.
- **It builds the national pipeline.** Train-the-trainer means the pilot's curriculum becomes the national curriculum.

5.3 Accreditation route

DFC to map and pursue the relevant Nigerian accreditation pathway (professional/regulatory councils and technical-education bodies), with optional partnership to an international EMT/paramedic curriculum for credibility. *The exact accrediting bodies and requirements are an action item for the Workforce pillar to confirm at source.*

6. Standards, SOPs and 112 / NCC strengthening

The implementation layer behind Dr. Doubra's asks #2 and #3:

- **Author and run** triage algorithms, call-centre SOPs, ambulance protocols, and clinical stabilisation standards - as DFC standards ready for NEMSAS to adopt nationally.
- **Strengthen 112 / NCC:** operator-training curriculum, AI-enabled decision-support to cut the operator decision-burden, and a QA / supervision framework for the line.
- **Hospital directory + referral pathways** so dispatch routes patients to the right facility, not just the nearest.

These are the cheapest, fastest, lowest-risk deliverables (Tier 0) and the ones DFC can publish and operate immediately, MOU or not.

7. Pooling resources and leveraging private-sector contributions

This is the resourcing philosophy that makes the whole thing affordable and is the concrete meaning of DFC as "the private-sector coalescing body."

What gets pooled:

Resource	How DFC pools it
Ambulances	The aggregation model (Section 4) - idle private capacity instead of bought fleet
Funding	The stacked capital structure (Section 8) + a pooled emergency fund
Equipment	Shared trauma-bay equipping and bulk procurement across pilot hospitals
Hospital capacity	A shared directory + referral network across public and private facilities
Clinical expertise	DFC member clinicians for standards, QA and training (internal, not a funder-facing line)
Data	One dispatch / outcomes dataset across all operators - the asset funders and government want

Leveraging private-sector contributions: CSR from banks, oil & gas and telcos; corporate ambulance fleets; private hospitals; HMOs and insurers. DFC convenes these into a coordinated whole that government can plug into through a single relationship. The private sector becomes the *engine*; government provides the statutory demand and funding rails.

8. How we raise the funding - a stacked capital structure

The meeting's sharpest line: "*free care still requires someone to pay for it,*" and "*government funding alone is unreliable.*" The answer is a **stack**, each layer doing a different job:

Layer	Source	Funds	Why it fits
Catalytic / grant	Gates, PSHAN, World Bank, CSR (banks, oil & gas, MTN)	One-time capex: training, tech build, equipping, the pilot	Funders buy startable, measurable pilots, not recurring care
Statutory / public	BHCPF (EMT Gateway → NEMSAS), NHIA Vulnerable Group Fund	Recurring per-case stabilisation for indigent / uninsured	What the EMS Act + tariff-authority advocacy <i>secures</i>
Earned revenue	Training & certification fees; aggregator take-rate; B2B cover	Ongoing operations - the self-funding core	DFC's own engine, not dependent on anyone
Insurance pooling	HMOs, LASHMA / state schemes	Per-case cost for insured patients through the same system	Turns the system into a billing channel, not a cost centre
Consumer / micro	USSD micro-levy (Safemama *3581# / ₦50 precedent), telco partnership	Sustains the 112 comms layer	Validated in-market; cheap; scales with usage

The insight for the group: *grants buy the build; earned revenue (training + aggregation) plus the statutory and insurance layers buy the sustainability.* The narrow advocacy ask exists precisely to lock the statutory layer.

Per the core-team note, DFC in-kind member time is **omitted** from the funder-facing budget. It stays internal/governance only, so we never imply a free subsidy that undercuts the sustainability case.

9. How we implement - with AND without government

The most important design decision and the one that de-risks everything. **Every workstream has a "no-MOU path."** Government alignment is an *accelerant*, *not a dependency* - the only sane posture given NEMSAS runs on an executive order with no guaranteed budget.

Workstream	WITHOUT government (DFC moves now)	WITH government (what the MOU adds)
Standards / SOPs	Publish as DFC standards; train DFC-network hospitals	NEMSAS adopts them as national SOPs

Workstream	WITHOUT government (DFC moves now)	WITH government (what the MOU adds)
Ambulance aggregation	Run the pool commercially for HMOs / corporates / self-pay in pilot LGAs	NEMSAS recognises the block; BHCPF reimburses trips; scales national
Training institution	Train and certify under DFC accreditation	Government recognises DFC certification nationally
112 / call handling	Run a DFC dispatch line / app for the pilot network	Integrate with and strengthen national 112 + NCC
Trauma fund	Pilot a donor/privately-funded "48h free" pool	NHIA / BHCPF assumes the recurring liability
Awareness	Run the community survey + awareness campaign	Government co-brands; amplifies via NOA

The posture this gives us with government is powerful rather than supplicant:

"DFC is already building this. The MOU lets you nationalise it and plug it into statutory funding. Without you it still saves lives in Lagos and FCT; with you it becomes the national system."

That is how DFC *earns* recognition as the private-sector coalescing body - by moving, not by asking.

10. Pilot footprint - Lagos + FCT, costed as a ladder

Per core-team direction, the budget is a **cost ladder** so the group picks the envelope at ratification. Because DFC **aggregates** private ambulances rather than buying a fleet (Section 4), the **asset-light** figures below are the recommended basis; the owned-fleet numbers are retained in the Costing Model only as an upper bound.

Tier	Footprint	Year-1 cost - asset-light (recommended)	Owned-fleet (upper bound)
Lean	1 LGA each in Lagos + FCT (2 sites)	≈ ₦0.47 bn (~US\$0.31 m)	₦0.70 bn
Standard	2-3 LGAs per city (~5 LGAs)	≈ ₦1.10 bn (~US\$0.71 m)	₦2.04 bn
Corridor	Cities + East-West road (hybrid)	≈ ₦3.25 bn (~US\$2.10 m)	₦4.38 bn

Trauma fund (Tier 2 add-on), sized separately: **≈ ₦0.4 bn / ₦1.0 bn / ₦2.0 bn per year** for Lean / Standard / Corridor. Full line-item build-up in the [Costing Model](#).

Site-selection principle (from the core team): operational merit, not politics. Lagos and FCT score highest - existing emergency structures to build on (LASAMBUS / LASEMA; federal density), best infrastructure, best chance of a scalable, measurable model. The East-West corridor is the Corridor-tier stretch goal.

11. Governance & the MOU

Two documents form the basis of a **tripartite MOU** between DFC, NEMSAS and the SWAP office, and of formal recognition of DFC as the private-sector coalescing body for emergency care:

- **Document A - this proposal:** positioning, the delivery stack, aggregation model, training institution, financing architecture, the with/without-government design, the recognition ask.
- **Document B - the pilot plan + Costing Model:** sites, deliverables, golden-hour metrics, timeline, tiered budget.

Proposed governance: - **Steering Committee** - DFC President, NEMSAS DG, SWAP Coordinator (tripartite oversight). - **DFC ERI Core Team** - pillar leads (Aggregation, Workforce/Training, Standards/Clinical, 112/Tech, Finance, Advocacy), chaired by the ERI Coordinator. - **Pilot delivery units** - one lead per pilot city, shared M&E reporting golden-hour metrics monthly. - **Independent M&E** - so the cost-effectiveness evidence is credible to funders and government.

12. Funding map (Finance pillar)

The proposals *cannot go out without this*. The Finance pillar owns it.

Source	What it can fund	Mechanism / access route	Indicative ticket	Owner
BHCPF - EMT Gateway	Recurring 48-hour emergency care; reimburses accredited private AND public ambulance providers	~5% of BHCPF via the EMT Gateway to NEMSAS (NHA 2014 §11); already funds NEMSAS free-48hr care	Per-case; tariff-dependent	<i>TBD</i>
NHIA - Vulnerable Group Fund	Cover for indigent emergency patients	Via NHIA / state schemes	Per-case	<i>TBD</i>
Gates Foundation	Catalytic capex: tech, training, M&E	Concept note → proposal	\$250k-\$2m	<i>TBD</i>

Source	What it can fund	Mechanism / access route	Indicative ticket	Owner
PSHAN	Coalition-building, Vivo Health linkage	Membership + programme alignment	Programmatic	<i>TBD</i>
CSR - banks / oil & gas / MTN	Ambulances, 112 comms, awareness	Direct corporate partnership; MTN tie to 112 routing	₦50m - ₦500m	<i>TBD</i>
HMOs / LASHMA	Per-case cost for insured patients	Routing + billing integration	Per-case	<i>TBD</i>
USSD micro-levy	112 comms sustainability	Telco partnership (Safemama precedent: ₦50 /run)	Micro, high-volume	<i>TBD</i>
Training & certification fees	Workforce pillar operations	DFC accredited programmes	Per-trainee	<i>TBD</i>
Aggregator take-rate	Dispatch coordination & QA	Per reimbursed dispatch	% of tariff	<i>TBD</i>

Finance pillar's first task: populate ticket sizes and owners, and confirm directly with NEMSAS exactly which costs the EMT Gateway reimburses today and at what tariff. (Our research found official descriptions of BHCPF's gateway structure are inconsistent; verify at source.)

13. The "move now" track - what DFC starts this quarter, MOU or not

Acting on our commitment that *DFC should move on all the earmarked initiatives pending government alignment*. None of the following needs anyone's permission:

1. **Community-awareness survey** (his own proposal): do people know the number, have they used it, did they get a response, do they trust it, what are the barriers. The single most powerful asset for partnership and funding conversations.
2. **Tier-0 standards pack:** publish DFC triage algorithms, call-centre SOPs and ambulance protocols.
3. **Ambulance-pool credentialing framework + first operator sign-ups** in the two pilot LGAs.
4. **Stand up the training institution track:** map accreditation, build the first EMT / operator / responder curricula, run a first cohort.
5. **112 operator-training curriculum** and a **hospital directory + referral pathways** for the pilot LGAs.

These generate the proof and data that make the government proposal undeniable, and they begin saving lives in the pilot LGAs regardless of when the MOU lands.

14. Immediate asks of the group

- **Pillar leads:** assemble deliverables, owners, timelines and indicative resources under your pillar. Use the [Costing Model](#) assumptions as starting line items and challenge them.
 - **Finance pillar:** populate the funding map (§12) with ticket sizes and owners, and confirm the BHCPF / EMT-Gateway reimbursement reality with NEMSAS. **Gating input.**
 - **Aggregation pillar:** begin the operator-credentialing framework and a first list of Lagos + FCT private operators to approach.
 - **Workforce pillar:** confirm the Nigerian accreditation pathway for an EMS training institution.
 - **All members:** input on which two LGAs (Lagos + FCT) for the Lean pilot, especially those with operational footprint there.
 - **Decision sought at the next core meeting:** ratify (a) the RTA wedge, (b) the positioning statement (§0), (c) the aggregation model as the flagship, (d) the training-institution ambition, (e) the pilot tier to fund first, and (f) green-light the §13 "move now" track.
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Companion: 01 - Costing Model & Evidence Base. Source citations and verification status are in that document.